

PERSONAL HEALTH SMART REPORT

Prepared for

Aditya

Basic Info

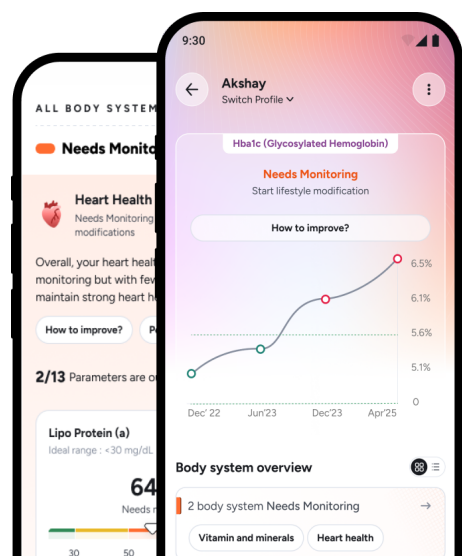
Male /25 Yrs

Sample collection date

12/05/2026

View detailed **health insights**, and trends on
Tata 1mg app.

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Doctor Summary

Cortisol, Serum (Morning Sample), Men Health Checkup Premium With Smart Report, Insulin, Fasting, Magnesium

Note This is an electronically generated **summary of clinically significant parameters of the attached report**. It is advised to read this summary in conjunction with the attached report and to correlate it clinically. For the trends section, the out-of-range values are highlighted with respect to the bio-reference range of respective reports.

Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)
Complete Blood Count	12 May 26		 Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!
Hemoglobin	15.8 g/dL	13.0 - 17.0	
RBC	5.25 10^6/cu.mm	4.5 - 5.5	
HCT	46.8 %	40 - 50	
RDW-CV	14.1 %	11.5 - 14	
Total Leucocyte Count	5.29 10^3/ÂµL	4 - 10	
Neutrophils	41 %	40 - 80	
Lymphocytes	39.1 %	20 - 40	
Monocytes	10.1 %	2 - 10	
Eosinophils	9.4 %	1 - 6	
Basophils	0.4 %	0 - 2	
Platelet Count	203 10^3/ÂµL	150 - 410	
PDW	20.4 fL	9 - 17	
Inflammatory markers	12 May 26		
Erythrocyte Sedimentation Rate	3 mm/hr	0.5 - 10	

Doctor Summary

Cortisol, Serum (Morning Sample), Men Health Checkup Premium With Smart Report, Insulin, Fasting, Magnesium

Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)
Inflammatory markers	12 May 26		 Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!
C-Reactive Protein (Quantitative)	1.10 mg/L	0 - 4.9	
Iron Studies	12 May 26		
Iron Serum	104 Åµg/dL	65 - 175	
Total Iron Binding Capacity (TIBC)	285 Åµg/dL	250 - 460	
Ferritin	137.75 ng/mL	21.81 - 274.66	
Diabetes Profile	12 May 26		
Glycosylated Hemoglobin (HbA1c)	4.8 %	4 - 5.6	
Glucose - Fasting	79 mg/dL	70 - 99	
Kidney Health	12 May 26		
Creatinine	1.02 mg/dL	0.6 - 1.2	
Uric Acid	3.7 mg/dL	3.7 - 7.7	
Sodium	138 mmol/L	136 - 145	
Potassium	4.30 mmol/L	3.5 - 5.1	
e GFR	105 mL/min/1.73m2		

Doctor Summary

Cortisol, Serum (Morning Sample), Men Health Checkup Premium With Smart Report, Insulin, Fasting, Magnesium

Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)
Lipid Profile	12 May 26		
Cholesterol - Total	146 mg/dL	<= 199.9	
Triglycerides	56 mg/dL	<= 149.9	
Cholesterol - HDL	49 mg/dL	>= 39.5	
Cholesterol - LDL	86 mg/dl	<= 99.9	
Non HDL Cholesterol	97 mg/dL	<= 129.9	
Cardiac Profile	12 May 26		
Lipoprotein (a)	11.40 mg/dL	0 - 29.99	
Homocysteine	7.46 Åµmol/L	5.46 - 16.20	
High sensitivity CRP	0.90 mg/L	0 - 3	
Apolipoprotein - A1	134.00 mg/dL	95 - 186	
Apolipoprotein - B	70.00 mg/dL	49 - 173	
Apolipoprotein B/A1 Ratio	0.52 Ratio		
Liver Health	12 May 26		
Bilirubin - Total	0.70 mg/dL	0.3 - 1.2	
Protein, Total	7.90 g/dL	6.4 - 8.3	



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Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)
Liver Health	12 May 26		
Albumin	5.00 g/dL	3.5 - 5.0	
Aspartate Transaminase (SGOT)	23 U/L	11 - 34	
Alanine Transaminase (SGPT)	17 U/L	0 - 45	
Alkaline Phosphatase	90 U/L	50 - 116	
Gamma Glutamyltransferase (GGT)	11 U/L	0 - 54.9	
Pancreas Profile	12 May 26		
Lipase	30.0 U/L	0 - 64	
Amylase	47 U/L	28 - 100	
Urine Routine & Microscopy	12 May 26		
Specific gravity	1.010	1.003 - 1.035	
pH	5.0	4.6 - 8	
Glucose	Negative	Negative	
Protein	Negative	Negative	
Ketones	Negative	Negative	
Pus cells	1-2 /hpf	0 - 5	



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Cortisol, Serum (Morning Sample), Men Health Checkup Premium With Smart Report, Insulin, Fasting, Magnesium

Test Name	Result	Bio. Ref. Interval	Trends (For last three tests)
Urine Routine & Microscopy		12 May 26	 Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!
Red blood cell	Nil /hpf	0 - 2	
Casts	Nil /lpf	Nil	
Crystals	Nil	Nil	
Calcium and Bone Health		12 May 26	
Calcium	9.4 mg/dL	9.1 - 10.4	
Vitamin D (25-OH)	15.0 ng/mL	30 - 100	
Phosphorus	4.60 mg/dL	2.5 - 4.5	
Vitamins & Minerals		12 May 26	
Vitamin B12	508.0 pg/mL	187 - 833	
Vitamin B9	8.10 ng/mL	3.1 - 20.5	
Thyroid Health		12 May 26	
T3, Total	0.97 ng/mL	0.35 - 1.93	
T4, Total	6.7 Åµg/dL	4.87 - 11.72	
Thyroid Stimulating Hormone - Ultra Sensitive	2.064 ÅµIU/mL	0.35 - 4.94	
Free T3	2.84 pg/mL	1.58 - 3.91	

Doctor Summary

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Test Name

Result

Bio. Ref. Interval

Trends (For last three tests)

Thyroid Health12 May 26

Free T41.01 ng/dL0.7 - 1.48

Arthritis Screening12 May 26

Rheumatoid Factor - Quantitative< 20.0 IU/mL0 - 29.9

Allergy Panel12 May 26

Immunoglobulin E (IgE) Total30 IU/mL<= 99

Reproductive Health12 May 26

Testosterone, total502.08 ng/dL240.24 - 870.68

Prostate Health12 May 26

Prostate Specific Antigen, total0.481 ng/mL0 - 4



Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!

PO No :PO10002301101-473



Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236087 / 16945254	Report Date	: 12/May/2026 03:13PM
Sample Type	: Whole Blood-EDTA	Report Status	: Final Report

HAEMATOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	15.8	g/dL	13.0-17.0	Spectrophotometry (Cyanide-free)
RBC	5.25	10 ⁶ /cu.mm	4.5 - 5.5	Impedance
HCT	46.8	%	40 - 50	Calculated
MCV	89.0	fL	83 - 101	Calculated
MCH	30.2	pg	27 - 32	Calculated
MCHC	33.9	g/dL	31.5 - 34.5	Calculated
RDW-CV	14.1	%	11.5-14	Calculated
Total Leucocyte Count	5.29	10 ³ /μL	4 - 10	Impedance
Differential Leucocyte Count				
Neutrophils	41	%	40-80	DHSS/Microscopy
Lymphocytes	39.1	%	20-40	DHSS/Microscopy
Monocytes	10.1	%	2-10	DHSS/Microscopy
Eosinophils	9.4	%	1-6	DHSS/Microscopy
Basophils	0.4	%	0-2	Impedance/Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	2.17	10 ³ /μL	2 - 7	Calculated
Absolute Lymphocyte Count	2.07	10 ³ /μL	1-3	Calculated
Absolute Monocyte Count	0.53	10 ³ /μL	0.2 - 1	Calculated
Absolute Eosinophil Count	0.5	10 ³ /μL	0.02 - 0.5	Calculated
Absolute Basophil Count	0.02	10 ³ /μL	0.02-0.1	Calculated
Platelet Count	203	10 ³ /μL	150-410	Impedance /Microscopy
MPV	10.8	fL	6.5 - 12	Calculated
PDW	20.4	fL	9 - 17	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
 DHSS : Double Hydrodynamic Sequential System Flowcytometry
 Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

This test has been performed at
TATA 1MG HYDERABAD
 Roxana Padela Estate, 2nd floor,
 M.No.8-1-366/1/RPE/201 to 204, Tolichowki,
 Hyderabad, Telangana - 500008

Dr. Fareeha Jaweed
 MBBS, MD (Pathology)
 Consultant Pathologist
 Reg. No - TSMC/FMR/05179

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Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236087 / 16945254	Report Date	: 12/May/2026 03:32PM
Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate	3	mm/hr	0-10	Modified Westergren

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosus; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyl dopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

This test has been performed at
TATA 1MG HYDERABAD
Roxana Padela Estate, 2nd floor,
M.No.8-1-366/1/RPE/201 to 204, Tolichowki,
Hyderabad, Telangana - 500008

Dr. K Madhuri
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 80149

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Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
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Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236087 / 16945254	Report Date	: 12/May/2026 04:40PM
Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
HbA1c (Glycosylated Hemoglobin)				
Glycosylated Hemoglobin (HbA1c)	4.8	%	4-5.6	HPLC (NGSP certified)
Estimated average glucose (eAG)	91.06	mg/dL		Calculated

Comment:

Interpretation: HbA1c%

≤5.6	Normal
5.7-6.4	At Risk For Diabetes
≥6.5	Diabetes

Adapted from American Diabetes Association.

Comments:

A 3 to 6 monthly monitoring is recommended in diabetics. People with diabetes should get the test done more often if their blood sugar stays too high or if their healthcare provider makes any change in the treatment plan. HbA1c concentration represent the integrated values for blood glucose over the preceding 8-12 weeks and is not affected by daily glucose fluctuation, exercise & recent food intake.

Please note, Glycemic goal should be individualized based on duration of diabetes, age/life expectancy, comorbid conditions, known CVD or advanced microvascular complications, hypoglycemia unawareness, and individual patient considerations.

Factors that interfere with HbA1c Measurement: Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements.

Factors that affect interpretation of HbA1c Measurement: Any condition that shortens erythrocyte survival or decrease mean erythrocyte age (e. g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c.

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1c result does not correlate with the patient's blood glucose levels.

- HPLC - High performance liquid chromatography

This test has been performed at

TATA 1MG HYDERABAD

Roxana Padela Estate, 2nd floor,
M.No.8-1-366/1/RPE/201 to 204, Tolichowki,
Hyderabad, Telangana - 500008

Dr. G Munni Krishna
MD Biochemist
APMC / FMR/84082

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Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236085 / 16945254	Report Date	: 12/May/2026 04:21PM
Sample Type	: Fluoride Plasma F	Report Status	: Final Report

BIOCHEMISTRY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
FBS (Fasting Blood Sugar)				
Glucose - Fasting	79	mg/dL	70-99	Hexokinase/G-6-PDH

Fasting Plasma Glucose (mg / dL)	2 hr plasma Glucose (mg / dL)	Diagnosis
99 or below	139 or below	Normal
100 to 125	140 to 199	Pre-Diabetes (IGT)
126 or above	200 or above	Diabetes

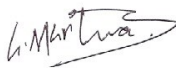
Reference : American Diabetes Association

Comment:

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

Plasma Glucose Goals	For people with Diabetes
Before meal	70-130 mg/dL
2 Hours after meal	Less than 180 mg/dL
HbA1c	Less than 7 %

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PO No :PO10002301101-473



Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236088 / 16945254	Report Date	: 12/May/2026 07:17PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid Profile				
Cholesterol - Total	146	mg/dL	Low (desirable): < 200 mg/dL Moderate (borderline) 200–239 mg/dL High: >= 240 mg/dL	Enzymatic
Triglycerides	56	mg/dL	Normal: <150, Borderline: 150 - 199, High:200-499, Very High>=500	Glycerol Phosphate Oxidase
Cholesterol - HDL	49	mg/dL	High risk <40mg/dL Low risk>=40mg/dL	Accelerator Selective Detergent
Cholesterol - LDL	86	mg/dl	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >=190	Calculated
Cholesterol- VLDL	11	mg/dl	<30	Calculated
Cholesterol : HDL Cholesterol	3.0	Ratio	Desirable : 3.5-4.5 High Risk : >5	Calculated
LDL : HDL Cholesterol	1.75	Ratio	Desirable : 2.5-3.0 High risk : >3.5	Calculated
Non HDL Cholesterol	97	mg/dL	Desirable:< 130, Above Desirable:130 - 159, Borderline High:160 - 189, High:190 - 219, Very High: >= 220	Calculated

Comment:

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BIOCHEMISTRY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
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- Lipid results show analytical and biological variation; repeat testing may be recommended before diagnosis or treatment decisions.
- Indians lie at high risk of developing early (a decade earlier than western populations) and more severe cardiovascular disease (ASCVD); higher mortality. Dyslipidemia (abnormal lipid profile) affects nearly 80% of population.
- Total cholesterol** is the sum of all cholesterol in the blood, including HDL, LDL, VLDL, and remnants.
- LDL Cholesterol (LDL-C)**, is the main "bad" cholesterol that contributes to plaque buildup, increasing the risk of heart disease and stroke, typically calculated by the Friedewald formula. Direct LDL-C measurement by homogeneous enzymatic assays carried out when triglycerides >400 mg/dL or dysbetalipoproteinemia.
- High-density lipoprotein (HDL)** or "good" cholesterol is anti-atherogenic (protective). Low HDL-C is a cardiovascular risk factor; seen in almost two-third of Indians. Values above 60 mg/dl are considered protective.
- Triglyceride (TG)** are a key driver of CVD. Indians are especially prone to atherogenic dyslipidemia—high TG, low HDL-C, and high LDL-C—closely linked to diabetes, metabolic syndrome, and insulin resistance; making TG management crucial.
- Non-HDL-Cholesterol (non-HDL-C)** Non-HDL-C measures all plaque-forming lipoproteins and is vital to monitor in high-TG patients (e.g., diabetics, obese) and those on statin therapy.
- Lipid Association of India (LAI-2020) recommends:-**

- Screening of all Indians above the age of 20 years for CVD risk factors, esp. lipid profile.
- Identification of Major Risk factors: Age (male ≥ 45 years, female ≥ 55 years); Family h/o heart disease at younger age (<55 yrs in males, <65 yrs in female or before menopause), current smoking/tobacco use, High blood pressure, Low HDL (males <40 mg/dl and females <50mg/dl).
- Fasting not mandatory; both fasting and non-fasting lipid profiles are useful for screening in Indian patients.
- LAI identifies both LDL-C and non-HDL-C as risk factors and recommends LDL-C, non-HDL-C and Apo-B as targets of lipid-lowering therapy.
- Lifestyle changes are the first-line approach for managing and preventing dyslipidemia. Treatment in low-risk individuals is initiated only after 3 months of unsuccessful lifestyle modification.
- Testing for Apolipoprotein B(Apo-B), hsCRP, Lp(a) should be considered for patients in moderate risk group.

Treatment targets for lipid-lowering therapy for various ASCVD risk groups

Risk Group	Treatment targets		
	LDL-C, mg / dL (primary target)	Non-HDL-C, mg / dL (co-primary target)	Apo-B, mg / dL (secondary target)
Low-risk group	<100	<130	<90
Moderate-risk group	<100 (optional <70)	<130 (optional <100)	<90
High-risk group	<70	<100	<80
Very high-risk group	<50	<80	<65
Extreme-risk group- category A	<50 (optional ≤ 30)	<80 (optional ≤ 60)	<65
Extreme-risk group- category B	≤ 30	≤ 60	<50
Extreme-risk group- category C	10-15	40-45	-

Source: LAI (2024) Consensus Statement IV

- Per **NCEP Expert Panel (2011) guidelines**, universal dyslipidemia screening is advised at 9–11 years and repeated at 17–21 years. Screening before age 2 yrs is not recommended; from age 2 onward, selective screening is done for children with a family history of premature CVD or risk factors such as obesity, diabetes, or hypertension.

This test has been performed at

TATA 1MG HYDERABAD

Roxana Padela Estate, 2nd floor,
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Hyderabad, Telangana - 500008Dr. G Munni Krishna
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Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Note: Biological Reference Interval as per National Cholesterol Education Program (NCEP) ATP III and LAI guidelines				

LIVER FUNCTION TEST

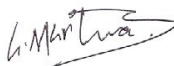
Liver Function Test

Bilirubin-Total	0.70	mg/dL	0.3-1.2	Diazonium Salt
Bilirubin-Direct	0.30	mg/dL	0-0.5	Diazo
Bilirubin-Indirect	0.40	mg/dL	0.2-0.8	Calculated
Protein, Total	7.90	g/dL	6.4-8.3	Biuret
Albumin	5.00	g/dL	3.5-5.0	Bromocresol Green
Globulin	2.9	g/dL	2.1 - 3.9	Calculated
A/G Ratio	1.72	Ratio	0.8 - 2.1	Calculated
SGOT (Aspartate Aminotransferase)	23	U/L	11-34	NADH w/o P-5'-P
SGPT (Alanine Transaminase)	17	U/L	0-45	NADH w/o P-5'-P
SGOT/SGPT	1.35	Ratio		Calculated
Alkaline Phosphatase	90	U/L	50-116	Para-Nitrophenyl Phosphate
Gamma Glutamyltransferase (GGT)	11	U/L	<55	L-gamma-glutamyl-3-Carboxy-4-Nitroanilide

Comment:

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis (e.g., gallstones, tumors).

This test has been performed at
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Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
• High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation, hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake, tissue damage, malabsorption, or increased renal excretion.				

Kidney Function Test with eGFR (18 Years & Above)

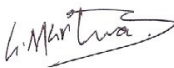
Blood Urea Nitrogen	18	mg/dL	8.9-20.6	Urease
Urea	38.52	mg/dL	19.0 - 44.0	Calculated
Creatinine	1.02	mg/dL	0.6-1.2	Kinetic Alkaline Picrate
Uric Acid	3.7	mg/dL	3.7-7.7	Uricase
Sodium	138	mmol/L	136-145	INDIRECT ISE
Potassium	4.30	mmol/L	3.5-5.1	INDIRECT ISE
Chloride	104.0	mmol/L	98-107	INDIRECT ISE
BUN/Creatinine Ratio	17.6	Ratio	12:1 - 20:1	Calculated
Glomerular Filtration Rate (estimated)	105	mL/min/1.73m ²		Calculated

- Calculation of eGFR is based on 2021 CKD-EPI creatinine equation, recommended method for estimating GFR in adults (>=18yrs). Estimates GFR using serum creatinine, age and sex.
- GFR categories defined acc. to KDIGO 2024 Clinical Practice Guideline for the Evaluation and Management of CKD.
- eGFR_{creat} levels less than 60 ml/min/1.73 m² should be considered as "decreased GFR".
- It is advised to estimate GFR using Cystatin C for confirmation of CKD in patients with eGFR_{creat} value 45-59 ml/min/1.73 sq.m without evidence of kidney damage and in specific circumstances when eGFR_{creat} is less accurate e.g. race, muscle mass, exercise, pregnancy, dehydration and nutritional status.

Interpretation:
GFR categories in CKD

Category	GFR (ml / min / 1.73 sq.m)	Terms
G1	≥ 90	Normal or high
G2	60-89	Mildly decreased
G3a	45-59	Mildly to moderately decreased
G3b	30-44	Moderately to severely decreased
G4	15-29	Severely decreased
G5	< 15	Kidney failure

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M.No.8-1-366/1/RPE/201 to 204, Tolichowki,
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PO No :PO10002301101-473



Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236088 / 16945254	Report Date	: 12/May/2026 07:17PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Criteria for CKD (Either of the following present for > 3 months)				
Markers of kidney damage (one or more)	Albuminuria (ACR >30 mg/g) Urine sediment abnormalities Electrolyte and other abnormalities due to tubular disorders Abnormalities detected by histology Structural abnormalities detected by imaging History of kidney transplantation			
Decreased GFR	GFR <60 ml/min/1.73 sq.m (GFR categories G3a-G5)			

Source: KDIGO 2024 Clinical Practice Guideline.

Calcium

Calcium 9.4 mg/dL 9.1 - 10.4 Arsenazo III

Comment:

Increased in: Hyperparathyroidism primary and secondary, Acute and chronic renal failure, Following renal transplantation, Osteomalacia with malabsorption, Acute osteoporosis, Malignant tumours (specially of breast, lung and kidney), Drugs: Vit. D and A intoxication, Diuretics, estrogen, androgen, tamoxifen, lithium

Decreased in: Hypoparathyroidism, Surgical and Idiopathic, Pseudohypoparathyroidism, Chronic renal disease with uremia and phosphate retention, Malabsorption of Calcium and Vit.D, obstructive jaundice, Bone Disease (Osteomalacia and rickets), Drugs: Cancer chemotherapy drugs, calcitonin, loop-actives diuretics, Hypomagnesemia,Hypoalbuminemia

Phosphorus, Serum

Phosphorus 4.60 mg/dL 2.5-4.5 Phosphomolybdate

Comment:

Phosphate metabolism is under the regulation of PTH, Vitamin D metabolites, and Fibroblast growth factor-23. Serum phosphate concentrations are about 50% higher in infants than in adults and decline throughout childhood as a consequence of the ability of growth hormone to increase the renal phosphate threshold.

Increased in:

Decreased Renal filtration (Acute or chronic renal failure) or increased reabsorption (e.g., hypoparathyroidism)
Increased Phosphate load (e.g., Oral or iv administration, Phosphate-containing laxatives or enemas, Vitamin D intoxication)
Cell Lysis (e.g., hemolysis, leukemias, chemotherapy, rhabdomyolysis)
Bone disease (e.g., healing fractures, multiple myeloma, Paget disease, osteolytic tumors)
Genetic (e.g., Hypoparathyroidism, Tumoral calcinosis)

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Test Name	Result	Unit	Bio. Ref. Interval	Method
Decreased in:				
Intracellular Shift (e.g., Oral or intravenous Glucose, Insulin, Diabetic ketoacidosis, Respiratory alkalosis, Alcoholism, Severe burns)				
Lowered Renal Phosphate Threshold (e.g., Primary or secondary hyperparathyroidism, Renal tubular defects)				
Decreased Intestinal Absorption (Malabsorption syndrome, Vitamin D deficiency) or increased loss (Vomiting, Diarrhea)				
Drugs (e.g., Salicylate, Paracetamol, Estrogens, Diuretics, Bisphosphonates, Anticonvulsants, Phosphate binding antacids, Antiviral drugs etc)				

Note:

Because a significant diurnal variation in plasma phosphate has been reported, fasting morning specimens are recommended. Levels are influenced by dietary intake, meals, and exercise.

Iron Studies, Comprehensive

Iron Serum	104	µg/dL	65 - 175	Ferene
Total Iron Binding Capacity (TIBC)	285	µg/dL	250-460	Calculated
Unsaturated Iron Binding Capacity	181	µg/dL	69 - 240	Ferrizone
Transferrin saturation	36.49	%	16-50	Calculated
Ferritin	137.75	ng/mL	21.81 - 274.66	CMIA

Comment:

Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron is seen in iron deficiency and anaemia of chronic disorders. Increased iron concentration are seen in hemolytic anaemias, hemochromatosis and acute liver disease. Serum Iron alone is unreliable due to considerable physiologic diurnal variation in the results with highest values in the morning and lowest values in the evening as well as variation in response to iron therapy .

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. Increased levels of TIBC suggest that total iron body stores are low, increased concentration may be the sign of Iron deficiency anaemia, polycythemia vera ,and may occur during the third trimester of pregnancy. Decreased levels may be seen in hemolytic anaemia, hemochromatosis, chronic liver disease, hypoproteinemia ,malnutrition.

Unsaturated Iron Binding Capacity (UIBC) is increased in low iron state and decreased in high iron concentration such as hemochromatosis. In case of anaemia of chronic disease the patient may be anaemic but has adequate iron reserve and a low UIBC.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

*Please note change in BRI of Ferritin.

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Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipase				
Lipase	30.0	U/L	<=64	kinetic colorimetric method

Comment:

Pancreas is the major and primary source of serum lipase, though lipase is also secreted by the gastric and intestinal mucosa. Lipase measurement in serum is used to diagnose acute pancreatitis. After an attack of acute pancreatitis, serum Lipase activity increases within 4 to 8 hours, peaks at about 24 hours, and decreases over 8 to 14 days. Concentrations often remain elevated longer than those of Amylase. The increase in serum Lipase activity is not necessarily proportional to the severity of the attack.

Increased levels are seen in:

- Acute & Chronic Pancreatitis.
- Obstruction of Pancreatic duct.
- Non pancreatic conditions like renal disease, intestinal obstruction, acute cholecystitis, duodenal ulcer, alcoholism, diabetic ketoacidosis and following endoscopic retrograde cholangiopancreatography (ERCP).

Amylase

Amylase	47	U/L	28-100	CMPG3 Substrate
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Comment:

- Amylase is a digestive enzyme mainly secreted by pancreas and salivary glands.
- An elevation of serum amylase beyond three times the upper limit of normal, combined with either clinical symptoms and/or imaging findings, may indicate acute pancreatitis. Serum amylase levels typically rise within 6 to 48 hours and usually return to baseline within 3 to 7 days. However, because of its short half-life, amylase levels may normalize as quickly as 24 hours after onset. Additionally, around 20% of patients with acute pancreatitis may have normal or near-normal amylase levels. Therefore, lipase, which is more specific to pancreatitis, should be measured alongside amylase to improve diagnostic accuracy.
- Elevated amylase levels can also be associated with conditions such as pancreatic duct obstruction, pancreatic carcinoma, or pancreatic pseudocysts. Additionally, increased amylase levels may occur in cholecystitis, renal disease, acute alcohol poisoning, following procedures like endoscopic retrograde cholangiopancreatography (ERCP) and even in non-pancreatic conditions like penetrating peptic ulcers, duodenal obstruction, mumps, ectopic pregnancy, and severe diabetic ketoacidosis.
- In asymptomatic individuals, elevated amylase levels may be attributed to macroamylasemia or idiopathic

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Test Name	Result	Unit	Bio. Ref. Interval	Method
hyperamylasemia; amylase levels may fluctuate. Transient increases in amylase may also result from inflammation, alcohol consumption, or medications such as aspirin, diuretics, oral contraceptives, corticosteroids, indomethacin, and opiates.				
• Low amylase levels are seen in chronic pancreatitis, congestive heart failure, 2nd & 3rd trimester of pregnancy, gastrointestinal cancer & bone fractures. Highly lipemic samples may show falsely low amylase levels.				

Lipoprotein (a)

Lipoprotein(a)	11.40	mg/dL	<30	Turbidimetry
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Comment:

Note: Lipoprotein(a)[Lp(a)] is considered an important risk factor for Coronary Heart Disease (CHD).

- * Lipoprotein (a) consists of an LDL particle that is covalently bound to an additional protein, apolipoprotein (a). Apo(a) has high-sequence homology with the coagulation factor plasminogen and, like LDL, Lp(a) contains apolipoprotein B100 . Thus, Lp(a) is both proatherogenic and prothrombotic. Lp(a) is an independent risk factor for CHD, Ischemic Stroke, and Aortic Valve Stenosis.
- * Lp(a) is highly heterogeneous molecule; the degree of atherogenicity of the Lp(a) particle may depend on the molecular size of the Lp(a)-specific protein.
- * Serum concentrations of Lp(a) are related to genetic factors, and are largely unaffected by diet, exercise and lipid -lowering pharmaceuticals. However, in a patient with additional modifiable CHD risk factors, more aggressive therapy to normalize these factors may be indicated if the Lp(a) value is also increased.

Usage:

Evaluation of increased risk for cardiovascular disease and events:

- * In individuals at intermediate risk for cardiovascular disease
- * In patients with early atherosclerosis
- * In patients with strong family history of early CHD

High Sensitive CRP

High sensitivity CRP	0.90	mg/L	Healthy Individuals: <= 3.0 Low Risk: < 1.0 Average Risk: 1.0 to 3.0 High Risk: > 3.0	Turbidimetry
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Comment:

Note:

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Test Name	Result	Unit	Bio. Ref. Interval	Method
1. Patients with persistently unexplained hs-CRP levels above 10 mg/L should be evaluated for other non-cardiovascular etiologies or sources of infection and inflammation.				
2. For cardiovascular risk assessment, hs-CRP should ideally be measured twice, two weeks apart, and the average value is used (accounts for within-subject variability), in metabolically stable patient.				

Interpretation:

- High-sensitivity C-reactive protein (hs-CRP) serves as a biomarker for cardiovascular risk assessment, typically evaluated in conjunction with the lipid profile.
- The American Heart Association and US Centers for Disease Control and Prevention have defined risk groups as follows: * <1.0 Low Risk; 1.0 - 3.0 - Average Risk; >3.0 High Risk
- Both hs-CRP and CRP measure the same protein, but hs-CRP is more sensitive and used for cardiovascular risk assessment. For accurate interpretation, testing should be done when the patient is in a stable, healthy state.
- Recent illness, infection, tissue injury, or other sources of inflammation may elevate hs-CRP and lead to an overestimation of cardiovascular risk; It should not be used in individuals with chronic inflammatory conditions (e.g., arthritis), as results may be misleading.
- Women on hormone replacement therapy have been shown to have elevated hs-CRP levels.
- Anti-inflammatory drugs (e.g., aspirin, ibuprofen, naproxen) and statins can lower CRP levels.

*Source: Pearson et al; Markers of inflammation and cardiovascular disease; Circulation, 107 (3) (2003)

Apolipoproteins A1 & B

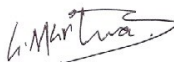
Apolipoprotein - A1	134.00	mg/dL	95-186	immunoturbidimetric
Apolipoprotein - B	70.00	mg/dL	49-173	immunoturbidimetric
Apolipoprotein B/A1 Ratio	0.52	Ratio		Calculated

Comment:

Apolipoprotein A1

- Apolipoproteins A1 (Apo A1) is the major apolipoprotein attached to HDL and is found in greater proportion than Apo A2 (3:1).
- It is inversely related to the risk of coronary artery disease (CAD).
- It may be a better predictor of atherogenic risk than HDL.

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Test Name Result Unit Bio. Ref. Interval Method

Apo A1 may be increased with	Apo A1 may be decreased with
Drugs (carbamazepine, estrogens, ethanol, statins,niacin, oral contraceptives, phenobarbital)	Chronic renal failure
Familial hyper alpha-lipoproteinemia	Coronary artery disease and peripheral vascular disease
Physical exercise	Drugs (androgens, beta blockers, diuretics and progestins)
Pregnancy	Familial hypo alpha-lipoproteinemia
Weight reduction	Smoking & Uncontrolled diabetes

Apolipoprotein B

- Apolipoprotein B (Apo B) is a major protein component of low density lipoprotein (LDL), Comprising >90% of the LDL. It is a more powerful independent predictor of coronary artery disease (CAD) than LDL cholesterol. It is useful in assessing the risk of CAD and to classify Hyperlipidemias.
- Apolipoprotein studies help in monitoring coronary bypass surgery patients with regard to risk and severity of restenosis.They are also useful in assessing risk of re-infarction in patients with Myocardial infarction .
- In patients with hyperapobetalipoproteinemia (HALB), a disorder associated with increased risk of developing CHD and with an estimated prevalence of 30% in patients with premature CAD, Apo B is increased disproportionately in LDL cholesterol. Apo B quantitation is used in distinguishing HALB from another common lipoprotein abnormality, Familial combined hyperlipidemia.

Apolipoprotein B:A1 Ratio

Elevated ApoB/ApoA1 ratio confers increased risk of atherosclerotic cardiovascular disease independently of LDL and HDL cholesterol concentrations.

Apo B to A1 ratio	
Ratio	Remarks
0.35- 0.98	Desirable
>0.98	Increased CAD risk

C-Reactive Protein Quantitative

C-Reactive Protein (Quantitative) 1.10 mg/L <5.0 Turbidimetry

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Test Name	Result	Unit	Bio. Ref. Interval	Method
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Comment:

- C-Reactive Protein [CRP] is an acute phase reactant ,hepatic secretion of which is stimulated in response to inflammatory cytokines.
- CRP is a very sensitive but nonspecific marker of inflammation and infection.
- The CRP test is useful in patient with Inflammatory bowel disease, arthritis, Autoimmune diseases, Pelvic inflammatory disease (PID), tissue injury or necrosis and infections.
- CRP levels can be elevated in the later stages of pregnancy as well as with use of birth control pills or hormone replacement therapy i.e. estrogen. Higher levels of CRP have also been observed in the obese.
- As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, the intensity of the rise being higher than ESR and the recovery being earlier than ESR. Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia.

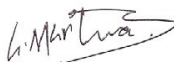
Rheumatoid Factor - Quantitative

Rheumatoid Factor - Quantitative	< 20.0	IU/mL	<30 Negative 30-50 Weak Positive >50 Positive	Immunoturbidimetric
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Comment:

- The detection of Rheumatoid factor (RF) is one of the criteria of the American Rheumatism Association (ARA) for the diagnosis of Rheumatoid Arthritis (RA).
- RF are heterogeneous group of auto antibodies directed against Fc- region of IgG molecules.
- They are useful in diagnosis of Rheumatoid Arthritis, but can also be found in other inflammatory diseases and in various non-rheumatic diseases.
- These occur in all the immunoglobulin classes, although the usual analytical methods are limited to the detection of Rheumatoid Factors of the IgM type. Healthy individuals >65 years of age may also show positive RF results.

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Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236090 / 16945254	Report Date	: 12/May/2026 04:34PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Magnesium				
Magnesium	2.02	mg/dL	1.60-2.60	Enzymatic

Comment:

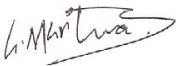
Magnesium (Mg) is an important cation essential for the function of more than 300 cellular enzymes.Total body Mg depends on GI absorption and renal excretion.50 - 60% of body magnesium content is stored in the bones.

Increased levels (Hypermagnesemia) : Acute & chronic renal failure, Addison's disease, Diuretics, antacids & laxative use, Hypothyroidism, Elderly diabetics.

Decreased levels (Hypomagnesemia) : Chronic nephritis, Acute pancreatitis, Alcoholic cirrhosis.



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IMMUNOLOGY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
Immunoglobulin E (IgE) Total	30	IU/mL	< 100	Turbidimetry

Comment:

- Immunoglobulin E (IgE) is the most important trigger molecule for allergic information.
- As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease.
- The level of IgE is low during the first year of life, gradually increases with age and reaches adult level after 10 years.

Uses

- For Allergy testing.
- Evaluation of children and adults suspected of having allergic respiratory disease
- To confirm clinical expression of sensitivity to foods in patients with Anaphylactic sensitivity or with Asthma, Angioedema or Cutaneous disease.
- To confirm the presence of IgE antibodies to certain occupational allergens

Increased Levels:

Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Bronchopulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases, Hodgkin's disease,etc.

Decreased Levels:

Hereditary deficiencies, Acquired immunodeficiency, Ataxia Telangiectasia, Non IgE Myeloma

Note:

Normal levels of IgE does not eliminate the possibility of allergic diseases
No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

Thyroid profile Total

T3, Total	0.97	ng/mL	0.35-1.93	CMIA
T4, Total	6.7	µg/dL	4.87-11.72	CMIA
Thyroid Stimulating Hormone - Ultrasensitive	2.064	µIU/mL	0.35-4.94	CMIA

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Comment:

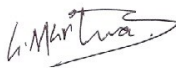
- Below mentioned are the guidelines for pregnancy related reference ranges for TSH, total T3 & Total T4.

Pregnancy			
	TSH (μ IU/mL) (as per American Thyroid Association)	Total T3 (ng/mL)	Total T4(μ g/dL)
1st trimester	0.1-2.5	0.81-1.90	7.33-14.8
2nd trimester	0.2-3.0	1.00-2.60	7.93-16.1
3rd trimester	0.3-3.0	1.00-2.60	6.95-15.7

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm .
- The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- TSH is secreted in a dual fashion: Intermittent pulses constitute 60-70% of total amount, background continuous secretion is 30-40%.These pulses occur regularly every 1-3 hrs.
- Total T3 & T4 concentrations are altered by physiological or pathological changes in thyroxine binding globulin (TBG) capacity .
- The determination of free T3 & free T4 has the advantage of being independent of changes in the concentrations and binding properties of the binding proteins.
- Changes in thyroid status are typically associated with concordant changes in T3, T4 and TSH levels.
- Unexpectedly abnormal or discordant thyroid test values may be seen with some rare, but clinically significant conditions such as central hypothyroidism, TSH-secreting pituitary tumors, thyroid hormone resistance, or the presence of heterophilic antibodies (HAMA) or thyroid hormone autoantibodies.
- For diagnostic purposes, results should be used in conjunction with other data.

TSH	T3	T4	Interpretation
High	Normal	Normal	Subclinical Hypothyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism

This test has been performed at
TATA 1MG HYDERABAD
Roxana Padela Estate, 2nd floor,
M.No.8-1-366/1/RPE/201 to 204, Tolichowki,
Hyderabad, Telangana - 500008


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MD Biochemist
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PO No :PO10002301101-473



Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236088 / 16945254	Report Date	: 12/May/2026 07:01PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
High	High	High	Secondary Hyperthyroidism	
Low	High/Normal	High/Normal	Hyperthyroidism	
Low	Low	Low	Non thyroidal illness / Secondary Hypothyroidism	

Free T4 1.01 ng/dL 0.7-1.48 CMIA

Comment:

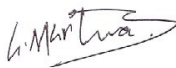
- Below mentioned are the guidelines for pregnancy related reference ranges for free T4.

Pregnancy	Reference Ranges(ng/dL)
1st trimester	0.7-2.0
2nd trimester	0.5-1.6
3rd trimester	0.5-1.6

- FT4 is the biologically active fraction of thyroxine in circulating blood.
- In patients with hyperthyroidism, the FT4 concentration increases, whereas in patients with hypothyroidism it generally decreases.
- Patients on hormone replacement therapy may have an elevation of FT4, although clinically they are euthyroid.
- The determination of free T4 has the advantage of being independent of changes in the concentrations and binding properties of the binding proteins.
- For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

TSH	T3 / FT3	T4 / FT4	Interpretation
High	Normal	Normal	Subclinical Hypothyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
High	High	High	Secondary Hyperthyroidism
Low	High/Normal	High/Normal	Hyperthyroidism

This test has been performed at
TATA 1MG HYDERABAD
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MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Low	Low	Low	Non thyroidal illness / Secondary Hypothyroidism	

Free T3 2.84 pg/mL 1.58-3.91 CMIA

Comment:

- Below mentioned are the guidelines for pregnancy related reference ranges for free T3.

Pregnancy	Reference Ranges(pg/mL)
1st trimester	2.0-3.8
2nd trimester	2.0-3.8
3rd trimester	2.0-3.8

- Free T3 measurements support the differential diagnosis of thyroid disorders, are needed to distinguish different forms of hyperthyroidism, to identify patients with T3 thyrotoxicosis, monitoring of patients with hypothyroidism treated with Thyroxine and antithyroid agents.
- The determination of free T3 has the advantage of being independent of changes in the concentrations and binding properties of the binding proteins.
- For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

TSH	T3 / FT3	T4 / FT4	Interpretation
High	Normal	Normal	Subclinical Hypothyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
High	High	High	Secondary Hyperthyroidism
Low	High/Normal	High/Normal	Hyperthyroidism
Low	Low	Low	Non thyroidal illness / Secondary Hypothyroidism

This test has been performed at
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Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin D (25-OH)	15.0	ng/mL	Deficiency:< 20, Insufficiency:20-29, Sufficiency:30 - 100, Toxicity possible:> 100	CMIA

Comment:

- Vitamin D is a fat-soluble steroid prohormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis.
- Two forms of vitamin D are biologically relevant - vitamin D3 (Cholecalciferol) and vitamin D2 (Ergocalciferol).
- Both vitamins D3 and D2 can be absorbed from food but only an estimated 10-20perc. of vitamin D is supplied through nutritional intake.
- Vitamin D is converted to the active hormone 1,25-(OH)2-vitamin D (Calcitriol) through two hydroxylation reactions. The first hydroxylation converts vitamin D into 25-OH vitamin D and occurs in the liver. The second hydroxylation converts 25-OH vitamin D into the biologically active 1,25-(OH)2-vitamin D and occurs in the kidneys as well as in many other cells of the body.
- Most cells express the vitamin D receptor and about 3perc. of the human genome is directly or indirectly regulated by the vitamin D endocrine system.
- The major storage form of vitamin D is 25-OH vitamin D and is present in the blood at up to 1,000 fold higher concentration compared to the active 1,25-(OH)2-vitamin D. 25-OH vitamin D has a half-life of 2-3 weeks vs. 4 hours for 1,25-(OH)2-vitamin D. Therefore, 25-OH vitamin D is the analyte of choice for determination of the vitamin D status.
- Risk factors for vitamin D deficiency include low sun exposure, inadequate intake, decreased absorption, abnormal metabolism, vitamin D resistance and liver or kidney diseases.
- Vitamin D deficiency is a cause of secondary hyperparathyroidism and diseases resulting in impaired bone metabolism (like rickets, osteomalacia).
- Recently, many chronic diseases such as cancer, high blood pressure, osteoporosis and several autoimmune diseases have been linked to vitamin D deficiency.
- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D

Utility Quantitative determination of 25-hydroxyvitamin D (25-OH vitamin D).

Vitamin B12

This test has been performed at
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IMMUNOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin B12	508.0	pg/mL	187-833	CMIA

Comment:

- Vitamin B12** along with **folate** is essential for DNA synthesis and myelin formation.
- Decreased levels** are seen in anaemia, term pregnancy, vegetarian diet, intrinsic factor deficiency, partial gastrectomy/ileal damage, celiac disease, oral contraceptive use, parasitic infestation, pancreatic deficiency, treated epilepsy, smoking, hemodialysis and advanced age.
- Increased levels** are seen in renal failure, hepatocellular disorders, myeloproliferative disorders and at times with excess supplementation of vitamins pills.

Vitamin B9 (Folic Acid)

Vitamin B9 (Folic Acid)	8.10	ng/mL	3.1-20.5	CMIA
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Comment:

Folate plays an important role in the synthesis of purine & pyrimidines in the body and is important for the maturation of erythrocytes. It is widely available from plants and to a lesser extent organ meats, but more than half the folate content of food is lost during cooking. Folate deficiency is commonly prevalent in alcoholic liver disease, pregnancy, and the elderly. It may result from poor intestinal absorption, nutrition deficiency, excessive demand as in pregnancy or in malignancy, and in response to certain drugs like Methotrexate & anticonvulsants. It is now routine practice to recommend dietary folate supplements from conception to the 12th week of pregnancy; such supplementation has been proven to reduce the incidence of neural tube defects.

Decreased Levels: Megaloblastic anemia, Infantile hyperthyroidism, Alcoholism, Malnutrition, Scurvy, Liver disease, B12 deficiency, dietary amino acid excess, adult Celiac disease, Tropical Sprue, Crohn's disease, Hemolytic anemias, Carcinomas, Myelofibrosis, vitamin B6 deficiency, pregnancy, Whipple's disease, extensive intestinal resection, and severe exfoliative dermatitis.

Note:

Certain drugs like Pyrimethamine, methotrexate, and trimethoprim are all folate antagonists i.e. they stop the action of the folic acid; phenytoin can decrease the intestinal absorption of folates, and ethanol both decreases absorption and increases excretion of folic acid.

To differentiate vitamin B12 & folate deficiency, measurement of Methylmalonic acid in urine & serum Homocysteine level is suggested.

Homocysteine

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Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Homocysteine	7.46	μmol/L	5.46-16.20	CMIA

Comment:

Interpretation:

Increased levels are seen in deranged Vit B12 metabolism and form an independent marker for risk of thromboembolic episodes in coronary artery disease (CAD)

Clinical Utility:

- Determine risk for heart disease, stroke and peripheral arterial blood vessel disease.
- Identify vitamin B12 deficiency or folic acid deficiency.
- Identify homocystinuria

The recommended use of Homocysteine (HCY) to assess risk factor for CAD are

- It is specially useful in young CAD patients (<40 years)
- In known cases of CAD,high HCYlevels should be used as a prognostic marker for CAD events and mortality.
- CAD patients with HCY levels >15 μmol/L belong to high risk group.
- Increased HCY levels with low vitamin concentrations should be handled as a potential vitamin deficiency case .

High values of HCY are found in dietary deficiency of folic acid, vitamin B6, or vitamin B12, homocystinuria, chronic liver and renal failure,post menopausal state , hypothyroidism, Alzheimer's disease,vaious neoplastic disease like cancers of ovary or breast and Acute lymphoblastic leukemia,drugs (anti-anticonvulsants, antibiotics, theophylline, birth control pills, and tamoxifen),alcoholism, smoking or tobacco usage.

Low values may be caused by some medicines or vitamins such as folic acid, vitamin B12, or niacin.

- Please note test values may vary depending on the assay method used.
- CLIA-Chemiluminescent Immunoassay

Testosterone Total

Testosterone, total	502.08	ng/dL	240.24-870.68	CMIA
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Comment:

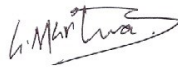
Stage	Male	Female
Tanner Stage I	<7.0 - 13.06 ng/dL	<7.0 - 10.06 ng/dL
Tanner Stage II	<7.0 - 79.13 ng/dL	<7.0 - 30.11 ng/dL
Tanner Stage III	<7.0 - 499.18 ng/dL	<7.0 - 30.49 ng/dL
Tanner Stage IV	79.10 - 747.17 ng/dL	<7.0 - 35.19 ng/dL
Tanner Stage V	224.83 - 669.65 ng/dL	1.88 - 39.30 ng/dL

- Testosterone is the major androgen in males produced by the Leydig cells of the testes.In females, it is secreted by adrenal cortex and ovaries.

This test has been performed at

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Barcode ID/Order ID	: D31236088 / 16945254	Report Date	: 12/May/2026 07:01PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
In serum, testosterone is largely bound to a specific steroid hormone-binding globulin (SHBG) (60% and to albumin (38%), but it is the free				

hormone (2%) that is physiologically active.

- The total testosterone level measures both bound and free testosterone in the serum.

Increased in: Idiopathic sexual precocity, adrenal hyperplasia (boys), adrenocortical tumors, trophoblastic disease during pregnancy, idiopathic hirsutism, virilizing ovarian tumors, PCOD, arrhenoblastoma, virilizing luteoma, testicular feminization (normal or moderately elevated), hyperthyroidism, Cushing's Disease, drugs (anticonvulsants, barbiturates, estrogens, oral contraceptives).

Decreased in: Hypogonadism (primary and secondary), orchidectomy, Klinefelter syndrome, uremia, hemodialysis, hepatic insufficiency, ethanol, drugs (digoxin, spironolactone, acarbose), excessive exercise.

Note

- Free testosterone should be measured in symptomatic patients with normal total testosterone levels.
- In men, there is a diurnal variation in serum testosterone with a 20% elevation in levels in the evenings.
- Physiological episodic secretion of testosterone may lead to variation in serum levels.
- Time of day, age, sex, puberty, pre & post menopause have an influence on testosterone concentration.
- Please note test values may vary depending on the assay method used.

This test has been performed at
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Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236088 / 16945254	Report Date	: 13/May/2026 10:08AM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
CORTISOL, SERUM (MORNING SAMPLE)				
Cortisol (morning sample)	20.0	µg/dL	Morning : 5.27-22.45	CLIA

Comment:

- Cortisol is the major glucocorticoid hormone secreted by the adrenal cortex. Its physiological functions include regulation of carbohydrate metabolism and electrolyte & water distribution. Cortisol also has immunosuppressive and anti-inflammatory activity.
- Cortisol levels are regulated by adrenocorticotrophic hormone (ACTH), which is synthesized by the pituitary in response to corticotrophin-releasing hormone (CRH). CRH is released in a cyclic fashion by the hypothalamus, resulting in diurnal peaks (6 a.m.-8 a.m.) and nadirs (11 p.m.) in plasma ACTH and cortisol levels.
- Plasma Cortisol levels are highest in the morning, and concentrations decrease towards evening. Cortisol is best measured in the morning when evaluating for possible Adrenal Insufficiency and best measured in the afternoon or evening to differentiate normal and Cushing's Syndrome subjects. Diurnal rhythmicity of cortisol is increased by systemic disease and stress.
- Increased levels are associated with Cushing syndrome, Adrenal and Pituitary adenoma/carcinoma, ectopic ACTH production, glucocorticoid therapy, stress, depression, hypoglycemia and hyperthyroidism.
- Decreased levels seen in Addison's disease, generalized adrenal hypofunction or a defect in the metabolic pathway for cortisol biosynthesis.
- In Newborns, a transient rise in cortisol occurs immediately after delivery and become stable by about 1 week of age.

NABL certificate
and scope

This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla
Industrial Estate, New Delhi, Delhi 110020

Reema
Dr. Reema Agrawal
MBBS, MD (Pathology)
Consultant Pathologist
Reg No: 56096

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IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Insulin Fasting				
Insulin - Fasting	3.80	μU/mL	2.0 - 25.0	CMIA

Comment:

Insulin is peptide hormone secreted from Beta cells of pancreas and the secretion is regulated by primarily by the blood glucose levels. The levels should be measured with the concomitant blood glucose.

Insulin levels are most frequently ordered following an abnormal glucose test and/or when a patient has acute or chronic symptoms of hypoglycemia, such as sweating, palpitations, hunger, confusion, blurred vision, dizziness, fainting, and seizures (although these can be caused by other conditions along with low blood glucose)

Uses of Serum Insulin levels:

1. Evaluation of PCOD
2. Diagnosis of Insulinoma
3. Classification of Diabetes mellitus
4. Diagnosis of fasting hypoglycemia.
5. Insulin autoimmune syndrome.
6. To monitor the success of islet cell transplant.

Elevated levels of Insulin may be seen in:

1. Insulinoma,
2. Some Type II diabetic patients
3. Infantile hypoglycemia
4. Hyperinsulinism
5. Obesity
6. Cushing's syndrome
7. Oral contraceptives
8. Acromegaly
9. Hyperthyroidism

Decreased levels may be seen in :

1. Untreated Type I Diabetes mellitus

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IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
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2. Hypopituitarism.

Note:

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin

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Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Prostate Specific Antigen (Total) PSA				
Prostate Specific Antigen, total	0.481	ng/mL	<4	CMIA

Comment:

- Prostate-specific antigen (PSA) is a glycoprotein, produced by prostate gland, urethral lining and bulbourethral gland. Normally, PSA is secreted into the seminal fluid in high concentrations, very little present in blood.
- Increased levels of serum PSA are seen in benign prostatic hypertrophy, prostatitis or prostatic cancers.

Clinical Use:

- Screening method for prostate cancer- when used in conjunction with digital rectal exam (DRE) in men 50 years or older.
- Testing should begin at younger age in high-risk men or those with first-degree relative diagnosed at a younger age.
- Total PSA level 4.0 to 10.0 ng/ml is described as diagnostic 'gray zone' - in such cases the Free PSA to Total PSA ratio (%Free PSA) helps to determine relative risk of prostate cancer.
- Total PSA >= 10.0 ng/mL indicates high probability of cancer and prostate biopsy is recommended for diagnosis.
- Management of prostate cancer - detecting metastasis, persistent disease, tumor recurrence and response to therapy.

Note:

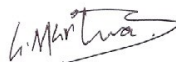
- PSA test should not be done immediately after cystoscopy, digital rectal examination (DRE), ejaculation, prostatic massage, indwelling catheterization, ultrasonography and needle biopsy of prostate as they can falsely elevate levels.
- PSA levels can be raised in patients with prostatitis, benign prostatic hyperplasia (BPH), urethral or prostatic trauma. Surgical castration or antiandrogen therapy can lower PSA levels dramatically.
- False negative / positive results observed in patients receiving mouse monoclonal antibodies for diagnosis or therapy or due to interference by heterophilic antibodies and nonspecific protein binding.
- PSA values regardless of levels should not be interpreted as absolute evidence of the presence or absence of disease. All values should be correlated with clinical findings and results of other investigations such as DRE.

Disclaimer:

Test values may vary depending on the assay method used. Values obtained with different assay methods cannot be used interchangeably.

CMIA-Chemiluminescent Microparticle Immunoassay /CLIA-Chemiluminescent immunoassay

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Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236089 / 16945254	Report Date	: 12/May/2026 01:38PM
Sample Type	: Urine	Report Status	: Final Report

CLINICAL PATHOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
Urine Routine & Microscopy				
Urine Routine	Manual			
Physical & Chemical Examination				
Colour	Pale Yellow		Pale Yellow	Manual
Appearance	Clear		Clear	Manual
Specific gravity	1.010		1.003 - 1.035	pKa change
pH	5.0		4.6 - 8.0	Double Indicator
Glucose	Negative		Negative	GOD-POD
Protein	Negative		Negative	Protein Error Principle
Ketones	Negative		Negative	Nitroprusside
Blood	Negative		Negative	Peroxidase
Bilirubin	Negative		Negative	Diazonium
Urobilinogen	Normal		Normal	Ehrlich
Leucocyte Esterase	Negative		Negative	Pyrrole
Nitrite	Negative		Negative	P-arsanilic acid
Microscopic Examination				
Pus cells	1-2	/hpf	0-5	Microscopy
Red Blood Cells	Nil	/hpf	0-2	Microscopy
Epithelial cells	2-3	/hpf	Few	Microscopy
Casts	Nil	/lpf	Nil	Microscopy
Crystals	Nil		Nil	Microscopy
Yeast	Nil		Nil	Microscopy
Bacteria	Nil		Nil	Microscopy

Comment:

- Note: Pre-test condition to be observed while submitting the sample-first void, mid stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum, Avoid prolonged transit time & undue exposure to sunlight.
- During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections. Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs.
- Urine microscopy is done in centrifuged urine specimens

This test has been performed at
TATA 1MG HYDERABAD
 Roxana Padela Estate, 2nd floor,
 M.No.8-1-366/1/RPE/201 to 204, Tolichowki,
 Hyderabad, Telangana - 500008

Madhuri

Dr. K Madhuri
 MBBS, MD (Pathology)
 Consultant Pathologist
 Reg. No: 80149

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PO No :PO10002301101-473



Customer Name	: Mr.ADITYA	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 25Y 1M 23D /Male	Referred By	: Dr.
Lab Visit ID	: HYD820783	Collection Date	: 12/May/2026 08:53AM
Barcode ID/Order ID	: D31236089 / 16945254	Report Date	: 12/May/2026 01:38PM
Sample Type	: Urine	Report Status	: Final Report

CLINICAL PATHOLOGY

MEN HEALTH CHECKUP PREMIUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
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*** End Of Report ***

Disclaimer:

1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
3. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

TATA 1mg Labs

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